

ST. JUDE NEUROLOGICAL INSTITUTE & ADVANCED BRAIN CLINIC

Department of Clinical Neuroscience • Relapsing-Remitting Multiple Sclerosis (RRMS) Comprehensive Evaluation

Patient Name:	Sophia Chen	Patient Code / MRN:	PT-7721
Date of Birth:	1981-11-25 (Age 44)	Gender:	Female
Primary Diagnosis:	Relapsing-Remitting MS (RRMS)	Attending Physician:	Dr. Sarah Jenkins, MD
Evaluation Date:	2026-08-22	Clinic:	Neuro-Immunology Center

CLINICAL BACKGROUND & SYMPTOM REVIEW

44-year-old female with established diagnosis of Relapsing-Remitting Multiple Sclerosis (RRMS) for 4 years. Patient reports current stability with resolved left optic neuritis episode from February 2026. Current complaints include intermittent bilateral lower extremity paresthesias (Lhermitte's sign positive) and moderate fatigue in the late afternoons. Expanded Disability Status Scale (EDSS): 2.0.

NEURO-IMAGING & CEREBROSPINAL FLUID (CSF) ANALYSIS

- Brain MRI with IV Gadolinium: Stable chronic periventricular and juxtacortical T2/FLAIR hyperintense demyelinating lesions. No new active enhancing lesions demonstrating disease stabilization.
- Cervical Spine MRI: Small non-enhancing lesion at C3-C4 dorsal column.
- Lumbar Puncture CSF Findings: Elevated IgG index (0.88, normal < 0.70), 6 discrete oligoclonal bands present in CSF not mirrored in serum.

DISEASE-MODIFYING THERAPY (DMT) & TOLERANCE

Patient currently maintained on Ocrelizumab (Ocrevus) 600 mg IV infusion every 6 months. Serum B-cell (CD19/CD20) suppression verified (< 1% total lymphocytes). Pre-infusion JC virus antibody index is 0.22 (Low Risk for PML). Liver function tests and immunoglobulin G levels remain within normal reference ranges.

PLAN & MANAGEMENT DIRECTIVES

Continue Ocrelizumab maintenance infusions every 24 weeks. Prescribed Amantadine 100 mg daily for MS-related fatigue. Routine physical therapy prescribed for gait optimization. Next scheduled MRI brain and cervical spine in 6 months.